

1. On day two of admission, an inpatient progress note states there is a stage three pressure ulcer of the sacrum that requires debridement. The coding professional composes a query to determine if this condition was present on admission (POA) by asking the physician if the pressure ulcer listed in the progress note of day two was present on admission—yes or no? Is that an acceptable query? Why or why not?

- a. No. Yes/no queries are not acceptable in any circumstances.
- b. No. Yes/no queries require clinical indicators.
- c. Yes. Yes/no queries may be used to established POA status.**
- d. Yes. Yes/no queries are the preferred query format for all queries.

2. Multiple choice queries must supply how many choices in order to be compliant?

- a. 3
- b. 4
- c. 5

d. No specific number

3. Compliant queries include which of the following?

- a. Comparison of reimbursement amounts
- b. Relevant clinical indicators**
- c. Potential impact on quality scores
- d. Impact on physician licensure

4. Examine the following query and determine if it is compliant and why:

Dr. Reynolds, is it possible that this patient has acute, postoperative blood loss anemia based on the drop in hemoglobin and hematocrit that occurred after surgery as noted in the lab report? Yes or no?

- a. Yes, because the query provides clinical indicators for the additional diagnosis.
- b. Yes, because the physician has the option to choose yes or no.
- c. No, because the query is leading and inappropriately includes the term “possible” in the question.**
- d. No, because yes and no queries are not acceptable.

5. Which of the following make a query compliant?

- a. Keeping the question vague so the physician has an opportunity to use his discretion when responding
- b. Explaining why the requested diagnosis is necessary to achieve a higher reimbursement
- c. Addressing the impact the query has on quality indicators
- d. Providing a concise presentation of facts and clinical indicators**

6. A patient was admitted with heart failure within one week of a heart transplant. Due to the timing, the coding professional thought the heart failure may indicate a transplant rejection. What action(s) should the coding staff take?

- a. Query the physician.**
- b. Assign the codes for the transplant rejection and the heart failure.
- c. Assign only the code for the transplant rejection.
- d. Assign only the code for heart failure.

7. A patient had a normal pregnancy and delivery with loose nuchal cord. Delivery, with birth of liveborn male infant, was accomplished with an episiotomy and repair. The delivery room record states "no evidence of fetal problem." What is the query opportunity for this record?

- a. Age of the patient
- b. Weeks of gestation/trimester**
- c. If there was adequate prenatal care
- d. If the pregnancy was high-risk

8. A toddler comes into the hospital admitted from the ER with the following: shortness of breath, wheezing, runny nose, and positive RSV test. The final diagnosis was viral infection upon discharge three days later. What condition should the coding professional query for in this scenario?

- a. Acute bronchiolitis**
- b. Acute bronchitis
- c. Croup
- d. Laryngitis

9. While admitted for an exacerbation of COPD, a patient developed swelling in the lower legs and had increasing shortness of breath despite the COPD treatment. An echocardiogram was performed

that showed an ejection fraction of 33 percent. A urinalysis showed albuminuria. Breathing treatments continued with the addition of Lasix to the medication regime. In the final diagnostic statement, the physician mentions only the COPD exacerbation. What is the query opportunity for this record?

- a. Coronary artery disease
- b. Acute congestive heart failure**
- c. Pleural effusion
- d. Atrial fibrillation

10. Which of the following condition combinations would benefit from a query?

- a. Hypertension and ESRD
- b. Diabetes and polyneuropathy
- c. CHF and hypertension**
- d. ESRD and diabetes

11. A patient was admitted with chest pain and shortness of breath. Preliminary lab work indicated nonelevated troponin with LBBB noted on EKG. Patient developed a fever and chills. X-ray finding demonstrated an infiltrate in the left lung. Sputum culture identified Klebsiella. The patient slowly improved after antibiotics were administered. Final discharge diagnosis listed as chest pain and patient was sent home on Piperacillin/tazobactam. For which diagnosis does a query opportunity exist for the principal diagnosis?

- a. Gram-negative pneumonia**
- b. Aspiration pneumonia
- c. Myocardial infarction
- d. Non-cardiac chest pain

Domain 4Regulatory Compliance

12. Authentication of health record entries means to:

- a. Create facsimiles of documents
- b. Prove authorship of documents**
- c. Develop documents
- d. Use a rubber stamp on documents

13. The requirements for documentation and record completion (documents such as history and physicals, discharge summaries, and consultations) as well as penalties for nonadherence must be specified in:

- a. Hospital rules and regulations
- b. Payer guidelines

c. Medical staff bylaws

- d. Nursing staff policies

14. Generally, data quality is defined as:

- a. Ensuring the greatest amount of data possible is obtained from the health record

b. Ensuring the accuracy and completeness of an organization's data

- c. Ensuring accuracy of the data collected for the case-mix index
- d. Ensuring the data for external reporting is optimized

15. The Joint Commission considers management that supports decision making to be important for safety and quality. What kind of management supports decision making?

- a. Resource management
- b. Risk management

c. Information management

- d. Case management

16. According to Medicare requirements, a history and physical must:

- a. Be coded based on the uniform hospital discharge proposal
- b. Include the patient's weight, height, body mass index, and year of birth

c. Be completed for each patient no more than 30 days before or 24 hours after admission or registration, but prior to surgery

- d. Discuss the educational plans for the patient including diet, exercise, and plans for smoking cessation

17. Medicare reimbursement depends on all of the following, except:

- a. The correct designation of the principal diagnosis

b. The number of codes that are assigned

c. The presence or absence of additional codes that represent complications, comorbidities, or major complications/comorbidities

d. Procedures performed

18. A coding professional reviews a health record and determines that a code Medicare has designated as “unacceptable principal diagnosis” is the correct code to assign. What should the coding professional do?

a. Assign another code from the history and physical as the principal diagnosis

b. Assign the code even though the insurer may not pay the claim

c. Use a comorbidity as the principal diagnosis

d. Assign a code from the outpatient visit prior to admission

19. A payer’s policy does not cover tetanus injections when provided as a preventive service but will cover them when provided as a postinjury service. If the injection is provided in the emergency department, what part of the claim will need to be modified to indicate the injection was a postinjury service rather than a preventive service?

a. Diagnosis code

b. Procedure code

c. Revenue code

d. Disposition code

20. The most challenging type of provider query is issued for:

a. Determining cause and effect

b. Establishing clinical validation

c. Resolving documentation conflict

d. Clarifying acuity or specificity

21. When creating a compliant query to clarify conflicting information from the surgeon and the attending physician, to whom should the query be directed?

a. Surgeon

b. Attending physician

- c. Medical staff director
- d. Medical records committee chairperson

22. When a compliant query remains unanswered, what is the next step for the coding professional?

- a. Ask the HIM director to place the physician on suspension until the query is answered.
- b. Alert the CEO that the query is outstanding, requesting a fine until the query is answered.

c. Refer to the internal escalation policy and follow the process outlined therein.

- d. Report the physician to the peer review committee for disciplinary measures.

23. When creating compliant queries coding professionals should:

- a. Query once without further follow up
- b. Query multiple times until the desired diagnosis is provided

c. Query once with additional follow up if necessary

- d. Query unlimited times until every discrepancy is resolved

24. Verbal queries:

- a. Are not permissible in any circumstance

b. Must have a written response in the health record for coding purposes

- c. Have different rules or criteria than written queries
- d. Are not required to be documented as long as the healthcare provider responds verbally

25. A patient is admitted with a high temperature, lethargy, hypotension, tachycardia, oliguria, and elevated WBC. The patient also has more than 100,000 organisms of Escherichia coli per cc of urine. The attending physician documents "urosepsis." What is the next step for the coding professional?

- a. Code sepsis as the principal with a secondary diagnosis of urinary tract infection due to E. coli.
- b. Code urinary tract infection with sepsis as a secondary diagnosis.

c. Query the physician to determine if the patient is being treated for sepsis, highlighting the clinical signs and symptoms.

- d. Ask the physician whether the patient had septic shock so that this may be used as the principal diagnosis.

26. A patient has findings suggestive of chronic obstructive pulmonary disease (COPD) on chest x-ray. The attending physician mentions the x-ray finding in one progress note but no medication, treatment, or further evaluation is provided. The coding professional should:

- a. Query the attending physician regarding the x-ray finding**
- b. Code the condition because the documentation reflects it
- c. Question the radiologist regarding whether to code this condition
- d. Use a code from abnormal findings to reflect the condition

27. If a patient undergoes an inpatient procedure and the final summary diagnosis is different from the diagnosis on the pathology report, the coding professional should:

- a. Code only from the discharge diagnoses
- b. Code the diagnosis reflected on the pathology report
- c. Code the most severe symptom
- d. Query the attending physician as to the final diagnosis**

28. A 56-year-old woman is admitted to an acute-care facility from a skilled nursing facility. The patient has multiple sclerosis and hypertension. During the course of hospitalization, a decubitus ulcer is found and debrided at the bedside by a physician. There is no typed operative report and no pathology report. The coding professional should:

- a. Use an excisional debridement code as these charts are rarely reviewed to verify the excisional debridement.
- b. Code with a non-excisional debridement procedure code.
- c. Query the healthcare provider who performed the procedure to determine if the debridement was excisional.**
- d. Eliminate the procedure code all together.

29. Patient presents with lower left quadrant abdominal pain with normal white cell count. X-ray showed sigmoid diverticulitis. Patient underwent a resection of sigmoid colon with anastomosis, developing a postoperative ileus after surgery. Nausea abated after resolution of the ileus. What is the query opportunity for this case?

- a. Was the diverticulitis perforated?
- b. Was the nausea postoperative?
- c. Was there an associated abscess with the diverticulitis?

d. Was the postoperative ileus a complication?

30. A 64-year-old female is admitted to the hospital with nausea, vomiting, and edema. The patient has a history of diabetes and takes Metformin and Lisinopril as prescribed. Blood sugar and blood pressure are monitored while admitted. On the discharge summary, the final diagnoses of acute renal failure and diabetes are documented. What is the query opportunity for this record?

a. Is the acute renal failure linked to the diabetes?

b. Does the patient have hypertension?

c. Does the patient have chronic renal failure?

d. Is the diabetes out of control?

31. A patient has documentation on the discharge summary of urosepsis. The coding staff queries the attending physician about the condition and is provided further information that the patient has septicemia. This is in alignment with the laboratory tests and medication given, but the diagnosis of septicemia was not documented by the physician. How should the physician be requested to document the septicemia?

a. A brand-new history and physical should be dictated to replace the one in the record.

b. An addendum to the chart should be written.

c. The new information should be squeezed in between lines within the progress notes of the last day.

d. The query sheet will be sufficient to document this information.

32. A patient was admitted with Type 1 diabetes with proliferative diabetic retinopathy to have surgery for traction retinal detachment for macular edema. Which of the following questions would make a compliant query for this patient?

a. Was the procedure performed on the left or right eye or bilateral eyes?

b. Is the retinopathy a complication?

c. Will you document use of insulin for this patient?

d. Is there a comorbid condition that can be documented to increase the reimbursement?

33. Compliant multiple-choice queries:

a. Must contain every possible option for the provider to choose from

b. Can provide a new diagnosis with supporting clinical indicators

c. Should provide a minimum of three options

d. Are the preferred query format for establishing POA status

34. Which of the following is a noncompliant query format?

a. Yes/no

b. Open-ended

c. Leading

d. Multiple choice

35. Identify if the following query is compliant and the appropriate rationale.

Dr. Jones, you state the patient has CHF in your discharge summary. Can you please indicate which type of the CHF the patient has?

_____ Acute on chronic

_____ Other

a. Yes, this is compliant query as it is in multiple choice format and the requirement is to provide at least two choices.

b. No, this is not a compliant query as it should have been an open-ended query format to allow the physician to identify the type of CHF.

c. No, this is not a compliant query as this is leading the physician. More reasonable choices should have been provided.

d. Yes, a multiple-choice query format is acceptable and referencing the diagnosed CHF makes this a compliant query.

36. An obstretic patient with a cephalic presentation and anticipating a vaginal delivery failed to progress after a trial of oxytocin. Measurement of the fetal head was performed, and the patient was immediately taken to surgery for a cesarean section. What condition should the coding professional suspect and query the physician about?

a. Twin pregnancy

b. Early delivery

c. Eclampsia

d. Cephalopelvic disproportion

37. A 45-year-old woman underwent a carotid bypass and experienced a significant drop in blood pressure during the surgery. The documentation suggested the patient may have had a myocardial infarction. In accordance with coding guidelines, what should the coding professional do?

- a. Code complication of surgery NOS.
- b. Query the physician to determine if the patient had hypotension.
- c. Query the physician to determine if there was a complication of surgery.**
- d. Code preoperative shock.

38. A patient's discharge summary does not contain a diagnosis that is documented by the anesthesiologist in a preoperative evaluation and that would impact MS-DRG assignment. The coding professional finds no evidence that the diagnosis contradicts documentation from the attending physician. The coding professional should:

- a. Code only from the discharge diagnoses
- b. Code the diagnosis reflected on the anesthesia preoperative evaluation**
- c. Code the most severe symptom
- d. Query the attending physician regarding the clinical significance of that diagnosis

39. When an inpatient has had multiple tests to evaluate an abnormal finding but no definitive diagnosis has been documented, the coding professional should:

- a. Assign a code for the abnormal finding without confirming with the physician
- b. Not assign any code for the abnormal finding
- c. Assign a diagnosis code based on the coding professional's judgment
- d. Query the physician regarding whether a diagnosis should be assigned or not**

40. A patient is admitted with Kaposi's sarcoma of the lung and a history of IV drug dependence is noted. Antiretroviral therapy is initiated for treatment with some alleviation of the patient's shortness of breath. Upon discharge, the only diagnosis listed was Kaposi's sarcoma of the lung. What other diagnosis might the coding professional expect to see with this condition and what action should be taken?

- a. CHF, add the code based on the "code also" note in the Tabular for the sarcoma
- b. HIV, add the code based on the "add additional code" note in the Tabular for the sarcoma
- c. CHF, query the physician based on the patient's shortness of breath and the "code first" note in the Tabular for the sarcoma

d. HIV, query the physician based on the previous drug dependence and the “code first” note in the Tabular for the sarcoma

41. A patient is admitted to the hospital with a high fever, chills, tachycardia, and a respiration rate of 26. Lactic acidosis is noted in the labs. Pneumonia is diagnosed and the patient admitted to ICU where antibiotics are given along with Levophed to help maintain the patient’s blood pressure which was in the extremely low range. The patient’s breathing was erratic and mechanical ventilation was initiated. With a final diagnosis of pneumonia, what is the query opportunity for this record?

a. Is this aspiration pneumonia?

b. Is this septic shock?

c. Is this Staphylococcus aureus pneumonia?

d. Is this sepsis?